THE MATERNITY METHOD

NCLEX CHEAT SHEET

Neonatal Abstinence Syndrome & Fetal Alcohol Syndrome

Maternal & Newborn › Newborn Complications

The Big Picture **NAS** (also called **NOWS** when opioids are the cause) = a newborn **withdrawing** from substances mom used in pregnancy. **FAS** = **permanent** damage from **alcohol** exposure before birth. Care is **nonjudgmental**: treatment and support help both mom and baby.

KNOW THE NUMBERS

24–48 hrs Withdrawal from **short-acting** opioids	**2–7 days** From **methadone/buprenorphi**	**0** **Safe** amount of alcohol in pregnancy	**Every 3–4 hrs** **Score** withdrawal (per tool)	**No** **Naloxone** for these babies

NAS: Spot It

- **High-pitched**, nonstop **cry**
- **Tremors**, stiff tone, **jumpy** Moro
- **Poor sleep** (< 1 hr after feeds)
- **Frantic sucking** but **poor feeding**; vomiting, **loose stools**
- **Sweating**, sneezing, yawning, stuffy nose, fast breathing, fever
- **Rubbed raw skin** on knees, nose, elbows

NAS: Assess

- **Finnegan** scoring, or **Eat, Sleep, Console** (ESC)
- **Daily weights**, I&O
- Skin breakdown, **dehydration**
- Mom's history, **urine/meconium/cord** drug screen (per policy)

Never give naloxone to a baby of an opioid-dependent mom: it can trigger **seizures**.

MEMORY HACK · COMFORT FIRST

The goal is a baby who can Eat · Sleep · Be Consoled

Swaddle snug, **flexed** position	**Quiet** **dim**, calm room; cluster care	**Parent** **room-in**, skin-to-skin, holding	**Suck** **pacifier**; small, frequent feeds	**Rock** slow, **vertical** rocking

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NAS: Treatment & Family Care

- **Nondrug care first** (above). Parents are the **best** treatment
- **Medications** if needed: **morphine** or **methadone** (or buprenorphine), slowly **tapered**; phenobarbital or clonidine as add-ons
- **Breastfeeding encouraged** if mom is stable in treatment (methadone/buprenorphine), no illicit drug use, and HIV-negative
- Mom: **don't stop opioids suddenly** in pregnancy. **Methadone/buprenorphine treatment** is recommended
- Be **nonjudgmental**; involve **social work**; teach safe sleep and soothing

FAS: Spot It

- **Face: smooth philtrum**, **thin upper lip**, **small eye openings**
- **Small head** (microcephaly), flat midface
- **Low birth weight**, poor growth
- Later: learning, attention, behavior and memory problems

FAS: Care

- **No cure**: damage is **lifelong**
- Early intervention, developmental **referrals**
- Baby may also **withdraw** from alcohol: tremors, irritability, seizures
- Teach: **no alcohol** at any point in pregnancy or while trying to conceive

RED FLAGS · STOP AND CALL THE PROVIDER

Seizures · **Weight loss** > 10% or dehydration · Rising withdrawal **scores** · **Apnea** · Signs that the **home is unsafe** → social work

MUST-KNOW FOR THE NCLEX

- **Comfort care first**: swaddle, quiet, holding, small frequent feeds, rooming-in.
- **No naloxone** for a newborn whose mom is opioid-dependent.
- Methadone/buprenorphine moms **can usually breastfeed**.
- **FAS face:** smooth philtrum, thin upper lip, small eye openings. No safe amount of alcohol.

QUICK CHECK · TEST YOURSELF

1. Withdrawal from methadone may not start until ____–____ days after birth.
2. Name 3 nondrug comfort measures for NAS.
3. Why avoid naloxone in a baby of an opioid-dependent mom?
4. The FAS lip finding is a _____ upper lip.
5. The "Eat, Sleep, _____" approach guides NAS care.

Answers: 1. 2–7 · 2. swaddle, quiet/dim room, skin-to-skin, pacifier, rocking, small frequent feeds · 3. It can cause sudden withdrawal and seizures · 4. thin · 5. Console

For study use only. Always follow facility protocols, current guidelines, and provider orders.